



NATIONAL HEALTH MISSION
Andhra Pradesh



35/10/2025

4,910 / 100

ICD Code:

L2 & L3 Facility - CHC/AH/MCH/DH/TH

Case Sheet for Maternity Services

LGSC REG No. 2910/25

Pinell 1.05 AM

wt 2.600 kg

Sex Male

1/12/25

HB1 - 102 gmm



watch station done 29/10/25

GP/BL/0 Ref

W/P

EDD 31/12/25

Name : Benchugadda Pallavi 25F / SC / Inddy

W/O or D/O : B. Praveen

IPD / Registration No. : 1624

Name of the facility : CHC Deoduluru

Cluster / Block : OBG & Gynaec

District : Eluru

Type: CHC/AH/MCH/DH/TH : CHC



Case Sheet for Maternity Services- L2 & L3 Facility



Admission Form

High risk/Complications: Yes/No

RCM No.	1280/2451384	Name of Facility	CHC, Dindur
Booked	Yes ☐ No ☒	Block	OBS & Gynaec
ICD/Registration No.	1627	District	EUPH
BPL/JSY Registration	Yes ☐ No ☐	Contact number (facility)	
Aadhar Card No.	923525087568	Name of ASHA	S. S. S. S.
Referred from:			
Referred reason:			

Name	Prasanthkumar, Pavan	Age	22F	W/O/R Do:	B. Pavan
Address	M. M. Pavan (U) Pula, Panchet (W), E. P. P.				
Contact No.	6308938029	Marital status	Married - Yes		
Admission date:	24/11/25	Time:	7:35 PM	Name of birth companion:	Prasanth
Admission category:	presented with labor pain referred in from other facility			presented with complications of pregnancy	
LMP	25/3/25	EOD:	1/1/25		
Provisional Diagnosis:	Gestational Diabetes	Final Diagnosis:	31/12/25		
Blood group & Rh type	A+ve	Injection Anti D given	Yes ☐ No ☐		

The following section is to be filled at delivery / discharge:

Delivery outcome: Live ☒ Abortion ☐	Sex of Baby: Male ☒ Female ☐
Fresh Still Birth ☐ Macerated Still Birth ☐	Preeclampsia: Yes ☐ No ☒
Single ☒ Twin/Multiple ☐	Birth weight (in kgs): 3.5 In VLUK ☐
Delivery date: 24/11/25 Time: 11:54 AM	Immunization: BCG ☐ OPV ☐ Hib ☐
Mode of Delivery/ Procedure: Normal ☐ Assisted ☐ CS ☒ Other (specify):	
Indication for assisted LSCS/ Others	PPH & 3rd stage of labor
Final outcome: Discharge / Death / LAMA / Abortion	
Referral during labour / Referral post delivery	
Name and signature of service provider: Dr. S. S. S.	Designation: CHC DSO
Phone No.	Date & Time

* (If preferred, attach the referral slip to case sheet)

Please tick the relevant details if Yes

Presenting complaints:

G2P1G1P0L1
34w 3d 10m

Past Obstetrics History

APH: ☐
PPH: ☐
MFE: ☐
C-section: ☐
Obstructed labor: ☐
Still births: ☐
Congenital anomaly: ☐
Anaemia: ☐
Others (Specify):

Medical/Surgical History
(Please specify)

Family H/o chronic illness:
(Please specify)

Date and time of onset of labor:

Gravida: G2

Parity: P1

Abortion: ☐

Living children: L1

History of Past Pregnancy

No.	Date of Delivery	Mom	Fetus	Place of Delivery	Normal / C-section	Weight of newborn	Any complication
1	35.8.2025	FBS	Live	G6F6L6W	C-section	2.5 Kg	NR (Angiomas)
2							
3							
4							
5							

General Examination

Height: 150 cm Weight: 51 kg BMI: 22.2

Palp: ☐ Jaundice: ☐ Facial Edema: ☐

Vitals

BP: 110/70 mmHg Temperature: 98.6 °C
Pulse: 88 /min Respiratory rate: 20 /min RR: 148 /min

PA Examination

Le: ☐ Right: ☒ Left: ☐ Others (Specify):

Gestational Age

Pre-term: ☐ Yes ☒ No ☐
LMP: 28.12.24 EDD: 11.12.25
Fundal height (in cms): 34.5
Age from USG: 36

Antenatal care received

Yes ☒ No ☐
Age from USG: 36

PV Examination

No. of PV Examinations: ☐
Cervical dilation: ☐ (in cms)
Mucous: ☐ Ruptured: ☐ Intact: ☐
Colour of amniotic fluid: Clear ☐ Green ☐ Blood ☐
Presents: ☐ Yes ☐ No ☐

27/12/2025

8004749

Consent for procedures

Extradental Follow

son/daughter/wife of

Parent Age (yr) 23/16

address Hills

Extracanal

myself

or other

age (yr)

relation (Son/Daughter/other/Mother/Wife/Other)

provide my consent for

Medical/Surgical

Anesthesia

procedure (Write the name of procedure)

I have been informed about the probable consequences of this procedure/anesthesia in detail, in the language I understand. I am signing this consent form without any undue pressure and in my complete consciousness.

The doctor/provider shall not be held responsible in case of any complication/misadventure.

Parent/Guardian's signature

For Family Planning

Consent for PPIUCD

Y. P. ROVEN

son/daughter/wife of

age (yr)

address

myself

or other

age

relation (Mother/Father/Husband)

provide my consent for procedure (Write the name of procedure).

I have been informed about the probable consequences of this procedure in detail, in the language I understand. I am signing this consent form without any undue pressure and in my complete consciousness.

The doctor/provider shall not be held responsible in case of any complication/misadventure.

Name & signature of patient

Name & signature of attendant

Robson Classification for Caesarean Section

Tick one Group as appropriate

Group 1  Multiparous with single cephalic pregnancy, >37 weeks gestation in spontaneous labour	Group 6  All multiparous women with a single breech pregnancy
Group 2  Multiparous with single cephalic pregnancy, >37 weeks gestation, indicated or were delivered by Caesarean section before labour	Group 7  All multiparous women with a single breech pregnancy, including women with previous uterine scars
Group 3  Multiparous without a previous uterine scar, with single cephalic pregnancy, >37 weeks gestation in spontaneous labour	Group 8  All women with multiple pregnancies, including women with previous uterine scars
Group 4  Multiparous without a previous uterine scar, with single cephalic pregnancy, >37 weeks gestation, who either had labour induced or were delivered by Caesarean section before labour	Group 9  All women with single pregnancy with a transverse or oblique lie, including women with previous uterine scars
Group 5  All multiparous with at least one previous uterine scar, with single cephalic pregnancy, >37 weeks gestation	Group 10  All women with single cephalic pregnancy, <37 weeks gestation, including women with previous scars

Notes for Caesarean Section Cases

Type: Emergency / Elective

Trial of labour done: Yes/No

Category:

- ☐ Immediate threat to life of woman or foetus
- ☐ Maternal or foetal compromise which is not immediately life threatening
- ☐ No maternal or foetal compromise but needs early delivery
- ☐ Delivery timed as suitable to woman/staff

Interaction complications, if any:
Notes

20/11/25

1st visit
stable
No soft
at 1st visit well.

Pl. No oral fluid
dab to healthy &

Complications During Delivery / Operation

Please tick relevant complications, if any:

Intra-partum for Vaginal Delivery		Intra-operative	
Complications to the Mother		Complications to the Mother	
1	Amniotic fluid embolism	1	Blood clot
2	Hematomas	2	Disruption of uterine arteries
3	Perineal laceration	3	Excessive blood loss (Haemorrhage)
4	Placental abnormalities (Abruption, Accreta, Retained placenta)	4	Hysterectomy
5	Pre-eclampsia	5	Incidental Surgical Injuries: Lacerations, abrasions and scarring
6	Prolonged labour	6	Patient Accreta
7	Obstructed Labour	7	Pelvic organ injuries: Uterine, Vaginal, Bladder and Bowel Injuries
8	Uterus Abnormalities (Ruptured or atony)	8	Uterine atony and Hypotonia
9	Unplanned hysterectomy		
Complications to the Baby		Complications to the Baby	
1	Birth Asphyxia	1	Birth Asphyxia
2	Birth Injury	2	Fetal Palsy
3	Cord Prolapse	3	Incidental Surgical Injuries: Lacerations, abrasions and scarring
4	ILFD (Intrauterine fetal death)	4	Intracranial haemorrhage
5	Meconium aspiration syndrome		
6	Shoulder dystocia		

Indication for CS
(Refer table below)
Other Notes:

PPV 38 week / 38 weeks

Please tick the relevant indications. You may choose multiple reasons.

SI no	Indication for CS	SI no	Indication for CS
1	Abdominal Cerebral	14	UGRI, Doppler compromise
2	Abnormal lie	15	Maternal request
3	Abnormal presentation: breech or transverse, oblique	16	Others (mention)
4	Abruptio Placenta	17	Pelvis tumours as a contraindication to delivery
5	Adherent Placenta	18	Placenta praevia
6	Bad obstetric history	19	Presumed fetal compromise - Abnormal CTG - Meconium stained liquor
7	Cephalo-pelvic disproportion (Previous LSCS for CPD)	20	Abnormal CTG + Meconium stained liquor Previous preterm caesarean section / hysterotomy / myomectomy / upper segment caesarean section
8	Cord prolapse / presentation	21	Previous uterine surgery (other than CS)
9	Failed induction	22	Rh ISO immunization
10	Failure to Progress	23	Suspected scar dehiscence
11	HELLP syndrome	24	Twin pregnancies (1 non cephalic)
12	Higher order multiples (triplets or more)	25	Two previous LSCS or more
13	HIV positive	26	Vaginal lesion / infection as a contraindication to vaginal delivery

Patient Name :
IP Number :
Chw Details:
Age :

SURGICAL SAFETY CHECKLIST

Before Induction of Anesthesia	Before Skin Incision	Before Patient leaves operating Room
a) Has the patient confirmed her identity? ☒ Yes ☐ No	a) Counter verified the patient with identity? ☒ Yes ☐ No	a) Surgeon and Anesthetist Plan of Recovery and management mention in the operating notes? ☒ Yes ☐ No
b) Is the site marked? ☒ Yes ☐ No	b) Confirm the procedure? ☒ Yes ☐ No	b) Count sponges and needles ☒ Yes ☐ No
c) Is the procedure consent verified? ☒ Yes ☐ No	c) Confirm when the incision to be made. ☒ Yes ☐ No	c) Specimen Label ☒ Yes ☐ No
d) Is surgical site prepared? ☒ Yes ☐ No	d) Has antibiotic prophylaxis been given within the last 60 mins. ☒ Yes ☐ No	d) Equipment sent for cleaning and sterilization ☒ Yes ☐ No
e) Is the jewelry removed? ☒ Yes ☐ No	e) Anticipated critical or non routine steps. ☒ Yes ☐ No	
f) Is the patient has any known allergies? ☒ Yes ☐ No	f) Turned the duration of the procedure? ☒ Yes ☐ No	
g) Is the IV line running? ☒ Yes ☐ No	g) Anticipated the amount of blood loss. ☒ Yes ☐ No	
h) Is the anaesthesia machine and medication arranged? ☒ Yes ☐ No	Anesthetist a) Assign patient safety concerns ☒ Yes ☐ No	
i) Is the Monitor checked? ☒ Yes ☐ No	Nursing Team a) Sterile field created? ☒ Yes ☐ No	
j) Is the blood or blood products arranged? ☒ Yes ☐ No	b) Arrange Sterile Peds ☒ Yes ☐ No	
	c) Is essential image displayed? ☒ Yes ☐ No	
Anesthesiologist Signature <i>Arif Per</i>	Surgeon's Signature <i>Arif</i>	O. T. Nurse's Signature <i>Arif</i>

NURSE'S DAILY RECORD

Name: E. Peltori Age: 24 Sex: F

Dept: OB&G Ward: OB&G

All injections to be initiated by Persons Administering and Witnessing

Hour	Medications	Diet	Treatment Remarks
0800			Case Council at 0800
0900			Feels 100% better
1000			BP: 100/60 mmHg
1100			HR: 140 bpm
1200			Feels 100% better
1300			BP: 100/60 mmHg
1400			HR: 140 bpm
1500			Feels 100% better
1600			BP: 100/60 mmHg
1700			HR: 140 bpm
1800			Feels 100% better
1900			BP: 100/60 mmHg
2000			HR: 140 bpm
2100			Feels 100% better
2200			BP: 100/60 mmHg
2300			HR: 140 bpm
0000			Feels 100% better

DELIVERY NOTES

Delivery date: 29/11/2025

Time: 07:06

Type of delivery:

Normal ☒

LSCS ☐

Others (Specify): ☐

Outcome:

Live birth ☒

Stillborn ☐

Others (Specify): ☐

Epidemiology:

Non-revised Still birth ☐

Single ☐

Yes ☐

No ☐

AMTBL performed:

Yes ☐

No ☐

1. Uterotonic administered:

Yes ☐

No ☐

2. OCT

Yes ☐

No ☐

3. Uterine massage

Yes ☐

No ☐

Complications, if any:

PPH ☐

Delayed labor ☐

Obstructed labor ☐

Maternal death ☐

Others (Specify): ☐

Causes and Time: ☐

Placenta inserted: ☐

Yes ☐

No ☐

BABY NOTES

Sex of the baby:

Male ☒

Female ☐

Birth weight:

2.6 kg ☒

Yes ☐

No ☐

Did the baby cry immediately after birth:

Yes ☒

No ☐

Did the baby require resuscitation:

Yes ☐

No ☐

Breastfeeding initiated:

Yes ☒

No ☐

Any congenital anomaly (please specify):

Yes ☐

No ☐

Any other complication (please specify):

Yes ☐

No ☐

Injection Vitamin K1 administered:

Yes ☒

No ☐

Vaccination done:

BCG ☒

OPV ☒

Hep B ☒

Temp of baby ☐

1st yell. done ☐

Identification for Baby

Applied Algebra 154/155 (2005) 69–82
Applied Algebra Group (Institute of Mathematics)
University of Warwick
Coventry CV4 7AL, UK
E-mail: maths@warwick.ac.uk

COT. M. Saitoh

2020 (March) / 94.6, 807 mg/dL
Fasting

Appendix 6.5 (Registration Agreement) : see the

[illegible]

Phylogenetic Position	Phylogenetic Position
Phylogenetic Position	Phylogenetic Position

pol. D. Pignatelli

W. H. S. 1875

Bf. 20160

And the

Prepared by: E. A. Sutherland

Date Time: 27/11/2025 09:43 AM

Operation/Procedure Notes (If applicable)

Procedure performed

Indication for the procedure

Whether Patient/Caregiver registered about the procedure and possibly

Consent of patient/ Guardian

Yes ☒ No ☐

Procedure end time: _____

Type of Anesthesia

Procedure notes

① Is well formed. ② Under bath.
③ digun clean and adequate on ground.
④ well. ⑤ under bath.

③ Abies latifolia delivoid by the Apron & 701
④ plants small even though surround

Condition at transfer to ward

4 days. Hemorrhoids cured. No more pain. No more itching. No more bleeding. No more trouble.

Still stable
On track for
2017

No such book is at present

Dea. Mrs. May
Dear Mother

② 41 for spring
③ 42 for summer

Dr. J. P. Buckley

⑦ $C_4' \cong M_{25}$.

Continuation Sheet for Post Delivery Notes

Any post delivery complication: Nil
(refer to the main report)

11/1/25

9 fac
vital signs stable.
No soft
uterine tenderness.
No no acute fluidy.
Vitals healthy.
Soft diet.
appetite

Post Delivery Complications

Please tick relevant complications, if any:

Normal/assisted Delivery		Caesarean Section	
Postpartum in Vaginal deliveries		Post-operative	
1	Eclampsia	1	Adverse reactions to medications, latex or anaesthesia
2	Hematoma	2	Delayed return of bowel function
3	Postpartum depression	3	Emergency hysterectomy
4	Postpartum Haemorrhage	4	Haemorrhage
5	Puerperal Infections	5	Incidental Hernia
6	Sepsis, Other infections (Wound infection, Mastitis, Puerperal Cystitis)	6	Sepsis/Infection (Peritonitis, Endomyometritis, UTI)
7	Thromboembolic disease	7	Scar tissue and difficulty with future deliveries
8	Thromboembolic disease (Deep ven, Pulmonary, Thrombophlebitis)	8	Thromboembolic disease (Deep ven, Pulmonary, Thrombophlebitis)
Complications to the Baby		Complications to the Baby	
1	Neonatal Sepsis	1	Neonatal Sepsis
2	Respiratory Distress Syndrome	2	Respiratory Distress Syndrome

Operation / Procedure Notes:

Incision : Joel Cohen's / Maylard's / Pfannenstiel / Paramedian / Midline / Vertical
 LUS : Performed / Thick / Stretched / Distended / Rupture
 UT Incision : Cervical / Inverted T / J Shaped / Extension LUS / Vertical / Oblique

Previous Scar : Yes / No (Type: _____)
 Adhesions : Yes / No (Type: _____)

Liquor Colour : Clear / Macerated / Blood stained
 Placenta : Upper / Lower / Fundal / Anterior / Posterior / Covering OS
 Presentation : Head / Breech / Shoulder / Footling / Cord / Trans / Oblique
 P.P. Position : Above rim / Deep in pelvis / Impacted

Vertical Uterine Tear : Yes / No
 Uterine Artery Involved: Right / Left / Both / Nil

Uterine Closure : Single Layer / Double layer
 Fibroids : Yes / No Type of Fibroid : Subserous / Submucous / Intramural

Uterine Anomaly : Number _____ Size _____
 : Unicorn / Bicorn / Septate / Didelphic / Arcuate / Others _____

Rt. Ovary : Normal / Absent / Cyst Tumour
 Lt. Ovary : Normal / Absent / Cyst Tumour

Rt. Tube : Normal / Absent / Cyst Tumour
 Lt. Tube : Normal / Absent / Cyst Tumour

Tubectomy : Yes / No If Yes, Right Tube Left Tube Method: _____
 HPE : Yes / No

Bladder Catheterized : Yes / No

Forceps : Yes / No
 Blood Loss : Minimal / 500ml / 500-1000 ml / more than 1000 ml / PPH > 1 Litre

Swab Count : 2 Sponges

Instruments Count : Nil

Placenta / cord abnormalities: None

Placenta Weight: 400 gm Sent for HPE: Yes / No

delg+r1

Agg. & D
m

Hypothetism ☐ Corollaries ☐ Death ☐

Date and time of transfer to PNC ward: 29 11 15 11:45 AM.
Condition at transfer to PNC ward: good
Special reason for referral of mother/baby:

good. 11:45 AM.

[illegible][illegible]

iQOO Z9x 5G
Dec 1, 2025, 11:33

Before Birth | SAFE CHILDBIRTH CHECKLIST

After: Read the questions on the left, then read the prompts on the right. The box on right for the condition present, then do the task and the appropriate box on the left.

Pregnancy No. 16, 17

CHECK-1 On Admission

Does Mother need referral?

- ☐ Yes, organized
- ☐ No

Parograph status?

- ☐ No, will start when < 4 cm
- ☒ NO, NO, OUTGOING unnecessary, independent operation at home

Does Mother need

- ☐ Antibiotics?
- ☐ Yes, given
- ☐ No

Is Magnesium Sulfate?

- ☐ Yes, given
- ☐ No

Corticosteroid

- ☐ Yes, given
- ☐ No

HIV status of the mother:

- ☐ Positive
- ☐ Negative

Yellow Universal Precautions

Are soap, water, gloves available?

- ☐ Yes, I will wash hands and wear gloves for each vaginal exam
- ☐ No, supplies arranged

Check if Mother or companion will call for help during labour if needed

- ☐ Yes, I will call for help if needed
- ☐ No, I will call for help if needed

Encourage a birth companion to be present during labour, at birth and all discharge

- ☐ Yes
- ☐ No

Name of Provider: Dr. Shafiq

Date: 20/1/25

Signature: [Signature]

Based on the WHO Safe Childbirth Checklist. <http://www.who.int/initiatives/repertoire-checklist-safely-deliver-at-home>

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Record temperature of mother: 36.5°C

Record BP of mother: 100/60 mmHg

Record fetal heart rate (FHR): 140 bpm

Record cervical dilation: 4 cm

Record uterine contractions: 3/10

Record fetal position: Head down

Record fetal presentation: Vertex

Record fetal lie: Longitudinal

Record fetal attitude: Flexed

Record fetal station: +1

Record fetal weight: 3.5 kg

Record fetal length: 48 cm

Record fetal head circumference: 34 cm

Record fetal chest circumference: 32 cm

Record fetal arm circumference: 30 cm

Record fetal leg circumference: 30 cm

Record fetal foot circumference: 25 cm

Record fetal hand circumference: 25 cm

Record fetal middle finger circumference: 25 cm

Record fetal thumb circumference: 25 cm

Record fetal index finger circumference: 25 cm

Record fetal middle ring finger circumference: 25 cm

Record fetal middle ring finger circumference: 25 cm

Record fetal middle ring finger circumference: 25 cm

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Record fetal middle ring finger circumference: 25 cm

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Record fetal middle ring finger circumference: 25 cm

Record fetal middle ring finger circumference: 25 cm

Record fetal middle ring finger circumference: 25 cm

CHECK 3 Soon After Birth (within 1 hour)

Record temperature of rectum
 Record pulse rate
 Record respiratory rate

Is Mother bleeding too much?
☐ Yes, about 100 ml
☒ No

Does Mother need:
☐ Assistance
☐ No

Is My Midwife/paramed satisfied?
☐ Yes, given
☒ No

Baby needs

Does Baby need:
☐ Assistance
☐ No

Is there any:
☐ Yes, given
☒ No

Special care and monitoring?
☐ Yes, organized
☒ No

Signs Newborn given
☐ Yes, given
☒ No

Signs Newborn given
☐ Yes, given
☒ No

Signs Newborn given
☐ Yes, given
☒ No

Signs Newborn given
☐ Yes, given
☒ No

Signs Newborn given
☐ Yes, given
☒ No

Signs Newborn given
☐ Yes, given
☒ No

Signs Newborn given
☐ Yes, given
☒ No

Signs Newborn given
☐ Yes, given
☒ No

Signs Newborn given
☐ Yes, given
☒ No

Signs Newborn given
☐ Yes, given
☒ No

Signs Newborn given
☐ Yes, given
☒ No

Signs Newborn given
☐ Yes, given
☒ No

Signs Newborn given
☐ Yes, given
☒ No

Based on the WHO Safe Childbirth Checklist, 2015. All rights reserved.

Just Before and During Birth | SAFE CHILD BIRTH CHECKLIST

Please fill out the questions on the left. Then, read the prompts on the right. The box on the right for the provider present, then fill in the box and the appropriate box on left.

Registration No.

CHECK 2 Just Before and During Birth (or C-Section)

Does mother react?

- ☒ Yes, green
- ☐ No

Is Myoglobinuria visible?

- ☒ Yes, green
- ☐ No

Common essential supplies are at

- ☒ Resuscitation room
- ☒ Suction
- ☒ Soap and clean water
- ☒ Oxygen to umbilic in syringe
- ☒ Pads for newborn

For baby

- ☒ Suction for upper trachea
- ☒ Suction for lower trachea
- ☒ Suction for mouth
- ☒ Suction for nose
- ☒ Suction for umbilic
- ☒ Suction for rectum

Stillborn assistant identified and ready to help at birth if needed?

- ☒ Yes
- ☐ No

Baby birth weight recorded?

- ☒ Yes
- ☐ No

*AMTSL - Inj. Oxytocin to umbilic M given within one minute of birth of baby?

- ☒ Yes
- ☐ No

Initiation of breastfeeding in first half-hour of birth of the baby

- ☒ Yes
- ☐ No

*AMTSL - Active Management of Third Stage of Labour

Name of Provider

Shah, Dr. Sybil D

Based on the WHO Safe Childbirth Checklist, 1999/2000, with updates to reflect current best practices and evidence. Adapted from the WHO Safe Childbirth Checklist, 1999/2000, with updates to reflect current best practices and evidence. Adapted from the WHO Safe Childbirth Checklist, 1999/2000, with updates to reflect current best practices and evidence.



Report temperature of mother
Report temp of mother: 38.0°C (100.4°F)
Report temp of baby: 36.5°C (97.7°F)

Give attention to Mother if any of the following are present:
• Mother's temperature: 38.0°C or 100.4°F
• Fetal heart rate: 150 bpm or more
• Fetal heart rate: 110 bpm or less
• Fetal heart rate: 150 bpm or more
• Fetal heart rate: 110 bpm or less
• Fetal heart rate: 150 bpm or more
• Fetal heart rate: 110 bpm or less

Use the box if it is required to take any of the following actions:
• Give attention to Mother if any of the following are present:
• Mother's temperature: 38.0°C or 100.4°F
• Fetal heart rate: 150 bpm or more
• Fetal heart rate: 110 bpm or less
• Fetal heart rate: 150 bpm or more
• Fetal heart rate: 110 bpm or less
• Fetal heart rate: 150 bpm or more
• Fetal heart rate: 110 bpm or less

Provide to baby for mother if any of the following are present:
• Give attention to Mother if any of the following are present:
• Mother's temperature: 38.0°C or 100.4°F
• Fetal heart rate: 150 bpm or more
• Fetal heart rate: 110 bpm or less
• Fetal heart rate: 150 bpm or more
• Fetal heart rate: 110 bpm or less
• Fetal heart rate: 150 bpm or more
• Fetal heart rate: 110 bpm or less

Provide to baby for mother if any of the following are present:
• Give attention to Mother if any of the following are present:
• Mother's temperature: 38.0°C or 100.4°F
• Fetal heart rate: 150 bpm or more
• Fetal heart rate: 110 bpm or less
• Fetal heart rate: 150 bpm or more
• Fetal heart rate: 110 bpm or less
• Fetal heart rate: 150 bpm or more
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